

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: A Phase III Study to Assess the Efficacy and Safety of NEXIUM for Maintenance of Healing of Erosive Esophagitis in Pediatric Patients 1 to 11 Years of Age **Short Title/Acronym:** Endoscopic Evidence of Maintenance of Healing With Oral NEXIUM in Patients 1 to 11 Years Old With Erosive Esophagitis **Protocol Number:** D9612C09998 **NCT Number:** NCT05267613 **Posting ID:** 850689877371742559 **Sponsor / Company Name:** AstraZeneca **Trial Status:** Recruiting **Investigational Product:** Nexium® (Esomeprazole magnesium) gastro-resistant granules for oral suspension **Phase of Development:** Phase III **Disease Area:** Erosive Esophagitis **Medical Monitor:** AstraZeneca Clinical Study Information Center

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To compare the safety and efficacy of two doses of Nexium in maintaining the healing of erosive esophagitis (EE) in pediatric patients 1 to 11 years of age. **Secondary Objectives:** To evaluate the overall safety and tolerability of Nexium via adverse event monitoring, clinical laboratory testing, vital signs, and physical examinations. To assess the percentage of days without the need for additional rescue medication.

2.2 Background & Rationale Esomeprazole (NEXIUM™) is indicated for the maintenance of healing of endoscopy-verified erosive esophagitis (EE) in children 1 to 11 years of age in a number of countries worldwide, but not currently in the United States (US). This study has been designed, in discussions with the Food and Drug Administration (FDA), to further evaluate the safety and efficacy of NEXIUM given as maintenance of healing of EE in children 1 to 11 years of age to address this regulatory and clinical gap.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator / Dose Comparison (No placebo group) **Blinding:** Double-blind (Masking applies to the randomized maintenance phase) **Randomization:** Randomized **Phase:** Phase 3 **Trial Status:** Recruiting

3.2 Planned Enrollment & Duration Targeted Enrollment: 50 pediatric patients **Number of Sites:** Approximately 47 sites

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Patient must be 1 to 11 years of age. 2. Patients must have a clinical history of GERD for at least 3 months before the start of the study. 3. For the healing phase: Patients must have confirmed presence of EE at endoscopy performed within one week of the start of the healing phase. 4. For the maintenance phase: Patients must have completed the healing phase and have endoscopy-verified healed EE at the 8-week endoscopy visit. 5. Patients must weigh ≥ 10 kg. 6. Patients may be male or female. 7. All postmenarcheal female patients must have a negative pregnancy test (urine) before starting treatment. 8. Sexually active patients must be abstinent or maintain effective contraception from informed consent day up to the last day of study intervention. 9. Patient's guardian must be capable of giving signed informed consent.

4.2 Exclusion Criteria 1. Presence of other diseases, such as severe heart, lung, liver, renal, blood, or neurological disease or similar. 2. Significant clinical illness within 4 weeks prior to the start of treatment. 3. Any conditions that are predicted to require a surgery during the study period (from the day of informed consent to the day of the last scheduled visit). 4. Previous total gastrectomy. 5. Anticipated need for concomitant therapy with PPIs (except for the IMPs), H₂-receptor antagonists and other outlined drugs after enrollment in this study. 6. Participation in another clinical study with an IMP administered in the last 4 weeks before enrollment. 7. Patients with a known hypersensitivity to NEXIUM, or any other PPI, or any of the excipients of the product. 8. Involvement in the planning and/or conduct of the study (applies to both AstraZeneca staff and/or staff at the study site). 9. Judgment by the Investigator that the patient should not participate in the study if the patient or guardian is unlikely to comply with study procedures, restrictions, and requirements. 10. Previous screening, or enrollment and randomization in the present study.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: 10 mg or 20 mg taken once daily for 8 weeks (healing phase), and then 10 mg or 5 mg taken once daily for 16 weeks (maintenance phase). **Route of Administration:** Oral (gastro-resistant granules for oral suspension). **Duration of Treatment:** 24 weeks total (8 weeks healing phase + 16 weeks maintenance phase).

5.2 Concomitant & Prohibited Therapy Permitted: Standard rescue medications as designated by the study protocol for GERD

breakthrough symptoms. **Prohibited:** Concomitant therapy with other Proton Pump Inhibitors (PPIs) and H2-receptor antagonists within 4 weeks prior to treatment or during the study.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	Screening	Pre-treatment Informed consent/assent, medical history (GERD > 3 months), negative pregnancy test (if postmenarcheal)
2	Healing Phase Start	Week 0 Endoscopy to confirm presence of EE, weight check, first dose of 8-week induction regimen
3	Maintenance Phase Start	Week 8 Endoscopy to verify healed EE, double-blind randomization to maintenance dose
4	End of Study	Week 24 Final endoscopy to assess maintenance of healing, end of 16-week maintenance phase, comprehensive safety/AE review

7. Outcome Measures (Endpoints)

7.1 Primary Outcome 1. Presence / absence of Erosive Esophagitis: Presence/absence of Erosive Esophagitis for all patients by assessment of Esophagogastroduodenoscopy at the end of the 16-week maintenance phase.

7.2 Secondary Outcomes 1. Presence / absence of Erosive Esophagitis. 2. Percentage of days without rescue medication. *(Additionally includes overall safety and tolerability profiles via monitoring of adverse events, clinical laboratory testing, vital signs, and physical exams).*

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring throughout the study including physical examinations, body weight tracking, hematology, clinical chemistry, urinalysis, blood pressure, and pulse. **Serious Adverse Events (SAEs):** Reported per standard pharmacovigilance timelines (typically 24 hours of site awareness). **Data Monitoring Committee (DMC):** Internal safety monitoring mechanisms to oversee pediatric compliance and safety.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for the primary maintenance of healing endpoint; Safety Set for all AE/SAE evaluations. **Sample Size Justification:** Targeted enrollment of $N=50$ adequately powered to assess comparative maintenance of EE healing and safety between the two double-blind pediatric maintenance doses. **Handling of Missing Data:** Standard imputation

(e.g., LOCF or Multiple Imputation) for dropouts or unreadable endoscopic evaluations.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring to ensure protocol adherence, medication accountability, and pediatric safety.

Audit Trail: eCRF locking and tracking of proper informed consent from guardians, and assent from pediatric participants where applicable.

11. Study Identifiers & Governance

Primary ID: D9612C09998 **Registry Number / NCT Number:** NCT05267613

Posting ID: 850689877371742559 **Sponsor/Lead Organization:**

AstraZeneca **Company:** AstraZeneca **Trial Status:** Recruiting **Study**

Title: Endoscopic Evidence of Maintenance of Healing With Oral NEXIUM in Patients 1 to 11 Years Old With Erosive Esophagitis

Official Regulatory Title: A Phase III Study to Assess the Efficacy and Safety of NEXIUM for Maintenance of Healing of Erosive Esophagitis in Pediatric Patients 1 to 11 Years of Age

12. Clinical Framework (Erosive Esophagitis Specific)

Primary Condition: Erosive Esophagitis (EE) / Gastroesophageal Reflux Disease (GERD) **Diagnosis Threshold:** Confirmed presence of EE at baseline endoscopy **Medical Methodology:** Acid Suppression Therapy (Proton Pump Inhibitor) **Optimization Target:** Maintenance of endoscopy-verified healed EE without relapse

13. Study Procedures & Methodology

Management Pathway: 8-week open-label healing phase followed by a 16-week randomized, double-blind maintenance phase. **Education**

Protocol (HCP): Pediatric endoscopy standardization and safety assessment training. **Education Protocol (Patient):** Guardian and

patient training on the proper preparation, measurement, and oral administration of the gastro-resistant granules suspension.

Quality Audit Mechanism: Medication adherence tracking and clinical laboratory test verification.

14. Observation & Follow-Up Schedule

Total Duration: 24 weeks of active treatment **Onsite Visit**

Cadence: Baseline (Week 0), Week 8, and Week 24 for core

endoscopic evaluations **Remote Monitoring Frequency:** Routine contacts for adverse event tracking and rescue medication usage. **Checklist Review Interval:** Continuous tracking to measure the percentage of days without the need for additional symptom medication.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					